MSC MARRIAGE AND FAMILY THERAPY

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MFT 701

Fundamentals of Marriage and Family Therapy

14 teaching weeks · 13 practicals · 16 worked cases · 7 figures

SEMESTER ONE

1.71 units · Prerequisite: none

> “This is the course that turns a helping professional into a systemic one.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 701 — Fundamentals of Marriage and Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	**Fundamentals of Marriage and Family Therapy**	One	1.71

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Course leader's summary. This is the course that turns a helping professional into a systemic one. It covers the intellectual history of family therapy, the epistemology that separates it from individual work, the profession's identity and scope, and the basic architecture of a first session. Everything later in the programme — theory, technique, assessment, ethics — assumes the vocabulary built here.

How to use this book

Each week contains four things: **the idea** (what you must understand), **the practice** (what you must be able to do), **the case** (what it looks like in a room), and **a reflective task** (what it asks of you personally). Read the week before the lecture; do the practical in the lab; write the reflection within 48 hours, while it is still uncomfortable.

The book is written to be read straight through, but it is also designed for revision: the integration chapter at the end compresses the whole course into two pages, and the glossary defines every term the examination may use.

1. Course description and rationale

Most mental health training teaches us to look inside a person: their thoughts, their symptoms, their attachment history. Family therapy teaches us to look between people: the loop, the triangle, the rule that nobody wrote down. This is not a rejection of individual psychology. It is an additional discipline of attention, and it changes what counts as the problem, the cause, and the cure.

Consider three common presentations:

Presentation	Individual formulation	Systemic formulation
A 15-year-old refuses school	Anxiety disorder, school phobia	A child whose symptoms hold a parental conflict at bay; the school refusal is the family's most effective way of preventing an argument about the marriage
A wife is "depressed"	Major depressive episode	A woman in a triad with her husband and his mother, carrying the family's unspoken grief about a lost pregnancy
A man "works too much"	Workaholism, avoidance	The person who regulates the family's emotional temperature by being absent, in a system that rewards his absence

None of these systemic formulations is automatically more true. Each is a hypothesis with different implications for where you intervene. The mature clinician holds both — the individual and the relational — and chooses deliberately.

Why this course first. Because students who begin with technique learn to do something they cannot explain, and students who begin with theory learn to explain something they cannot do. MFT 701 begins with framing: what kind of problem is this, and therefore what kind of help is required?

2. Learning outcomes

By the end of the course, students will be able to:

#	Outcome	Assessed in
LO1	Explain the epistemological shift from linear to circular causality, and its clinical consequences	Exam, Week 2 log
LO2	Trace the historical development of family therapy and locate contemporary models within it	Exam
LO3	Describe the profession's identity, scope of practice, and relationship to allied professions	Assignment
LO4	Conduct a structured first session, including joining, problem definition, and contracting	Skills assessment
LO5	Draw a three-generation genogram and generate hypotheses from it	Assignment
LO6	Critically discuss the common-factors evidence and its implications for model choice	Exam essay
LO7	Identify their own family-of-origin patterns and describe their effect on clinical work	Reflective log

3. Assessment

Component	Weight	Due
Reflective practice log (weekly, 300–400 words)	20%	Fortnightly
Genogram and hypothesis assignment (2,000 words + diagram)	30%	Week 9
Two-hour written examination (seen case + essay + short answers)	50%	Week 15

Skills gate. Students must pass a 15-minute observed first session (Week 6) before proceeding to MFT 706. The gate is pass/fail with one resit.

4. The fourteen-week teaching plan

Week	Topic	Practical	Core reading
1	What is family therapy? The relational turn	Triad: "the problem as described"	Nichols ch. 1; Handbook ch. 1
2	Linear and circular causality; first- and second-order cybernetics	Observation of a recorded argument	Watzlawick, Pragmatics ch. 1
3	A short history: from schizophrenia research to evidence-based practice	Timeline construction	Nichols ch. 2–3
4	Systems thinking: boundaries, feedback, homeostasis, rules	Feedback-loop mapping exercise	Becvar & Becvar ch. 2

Week	Topic	Practical	Core reading
5	The profession: identity, scope, settings, teamwork	Role boundaries case discussion	AAMFT code, scope sections
6	The first session I: joining, and the referral question	**Observed skills gate**	Minuchin & Fishman ch. 1
7	The first session II: defining the problem in the family's language	Triad: problem definition	Haley, Leaving Home ch. 1
8	The genogram as a hypothesis machine	Genogram lab (own family)	McGoldrick, Genograms ch. 1–2
9	Formulating relationally: the 5-P grid applied to a family	Formulation workshop	Dallos & Draper ch. 4
10	Culture, class and power in the consulting room	Reflexivity exercise	McGoldrick, Ethnicity & Family Therapy
11	Evidence: what works, for whom, and the common factors	Appraisal of one RCT and one qualitative study	Wampold ch. 2
12	Ethics, risk and the limits of the systemic frame	Risk-screen rehearsal	MFT 704 pre-reading
13	Integration I: three models applied to one family	Live demonstration + discussion	Nichols ch. 10
14	Integration II: your emerging clinical stance	Position statement presentation	Course review

Week 1 — What is family therapy? The relational turn

1.1 The lecture in ninety minutes

Week 1 has one job: to make the relational frame feel inevitable rather than eccentric. Everything a student does later — the first session, the genogram, the formulation, the ethics of conjoint work — assumes they can see a relationship as a thing with a shape, a history and a set of rules. Students arrive able to see people. Very few arrive able to see between people.

The ninety minutes run like this.

Minutes	Segment	What the lecturer does	What students do
0–10	Two formulations	Read the Okonkwo referral twice: once as an individual case, once as a relational one	Note which version they would have written
10–30	Where the field came from	The three failures of the individual frame (1.2)	Timeline exercise: place five founding texts
30–50	Five claims of the relational frame	Build the list on the board, testing each against a live example	Vote: which claim is hardest to accept?
50–65	Systems as a way of thinking	Cybernetics, feedback, the Watzlawick axioms (1.5)	Map one family loop on paper

Minutes	Segment	What the lecturer does	What students do
65–80	Three openings in a first session	Annotated transcript, read aloud in parts	Mark where the therapist widened the field
80–90	Set the lab and the journal	Hand out the widening-question protocol (1.11)	Form triads, choose a real (non-clinical) difficulty

The lecture is deliberately front-loaded with history. Students who know the field's origins stop treating systemic thinking as a technique among techniques and start treating it as an epistemology — a claim about what kind of thing a problem is.

1.2 Why the field exists: three failures of the individual frame

Family therapy was not invented because someone preferred groups to individuals. It was invented because three well-funded, well-intentioned research programmes ran into the same wall: the individual frame kept losing information that mattered clinically.

The first failure: the patient kept relapsing into a family. In the 1950s, Gregory Bateson's research group at Palo Alto — Bateson, Don Jackson, Jay Haley and John Weakland — was studying communication in families where one member had been diagnosed with schizophrenia. Their 1956 paper, "Toward a theory of schizophrenia," proposed the **double bind**: a pattern in which a person receives two messages at different logical levels that contradict each other, cannot comment on the contradiction, and cannot leave the field. A mother who says "come here, dear" while stiffening when the child approaches; a father who asks for honesty and then punishes it; a family that requires affection and forbids the expression of need that would make affection possible.

The double-bind theory has not survived as an explanation of schizophrenia. Its status as a causal claim about psychosis is dead, and students should know that. But something else in that research survived and became the spine of the field: the observation that **sequences of behaviour between people carry information that is invisible when you describe the people separately.** The patient's behaviour was intelligible as a move in a game; the game was not in the patient.

The second failure: the symptom made sense at home. Independently, researchers studying families of people with serious mental illness — Theodore Lidz at Yale, Lyman Wynne at the National Institute of Mental Health — noticed that families organised themselves around the symptomatic member. Wynne described **pseudo-mutuality**: a family that maintains the appearance of harmony by prohibiting any relationship that would threaten it. Lidz described **marital schism** and **marital skew**: two patterns in which the couple's conflict or the couple's imbalance shaped the child's development. Again the aetiological claims are dated. Again the structural insight held: **a family can be organised by a symptom**, and the symptom can be doing work for the system.

The third failure: the child was the family's messenger. The child guidance movement of the 1920s–1950s treated the child, advised the mother, and sent both home to a household that had not changed. Clinicians began to notice that improvement in the child was routinely followed by a new symptom in a sibling, or in the mother, or by a marital crisis that had been dormant. The pattern is now so familiar that it has a name —

symptom substitution — and the observation that produced it is the most practically useful idea in this week: if the symptom is functional for the system, removing the symptom without changing the system is unstable.

Three research programmes, three different populations, one conclusion: the unit of assessment was wrong. Between 1950 and 1975 the field built an alternative. Nathan Ackerman founded the Family Institute in 1960 and, with Jackson, the journal Family Process in 1962. Murray Bowen developed his theory of differentiation and multigenerational transmission at the Menninger Clinic and later at Georgetown. Virginia Satir was teaching conjoint family interviewing in California. Salvador Minuchin, working with delinquent boys at the Wiltwyck School and then at the Philadelphia Child Guidance Clinic, built structural family therapy out of the observation that these families were not sick so much as **organised at the wrong level of hierarchy** — children in charge, parents disengaged or in coalition, boundaries too diffuse or too rigid. Jay Haley joined Minuchin in Philadelphia in 1967 and brought strategy, power and the deliberate use of directives.

By the late 1970s the field had a canon: the Palo Alto communication school, Bowen's multigenerational theory, Minuchin's structural therapy, Haley and Madanes's strategic therapy, the Milan group's systemic therapy with its rituals and positive connotation, Satir's human validation, and the beginnings of solution-focused work at Milwaukee and narrative therapy in Adelaide and Auckland. Students will meet each of these properly in MFT 702. For now, one thing matters: **every one of these schools is an answer to the question "what is the unit?"** and each answers it slightly differently — the sequence, the generations, the structure, the meaning, the solution, the story.

> **DEBATE**
> "If the aetiological claims of the 1950s were wrong, the field built on them is unsound." Ten minutes, two sides. The strongest argument against: a science is allowed to keep its methods after discarding its first theories — medicine kept auscultation after humoral theory fell. The strongest argument for: the field has never fully shed the habit of blaming families for individual distress.

1.3 What "relational" actually means: five claims

"Systemic" is a word students learn to say before they learn to mean. Here are the five claims that make the relational frame a frame, in ascending order of how much they cost you as a clinician.

Claim 1 — Behaviour is patterned. Any two people who spend time together generate repeating sequences. The pattern is not in either person; it is in the loop. A couple who report "we never talk" will, within twenty minutes of a first session, demonstrate precisely how the not-talking is produced: one asks a question that contains an accusation, the other answers with a fact, the first escalates the volume, the second goes quiet. Neither is doing it to the other. Both are in it.

Claim 2 — The pattern is maintained by its consequences, not by its causes. This is cybernetics in one sentence. The question "why did this start?" is usually unanswerable and clinically useless; the question "what keeps it going?" is answerable and clinically decisive. A father's withdrawal is maintained by his wife's criticism, which is maintained by his withdrawal. Ten years of history explain how they got into the loop; only the loop explains why they are still in it.

Claim 3 — Meaning is negotiated between people. The same act means different things to different family members, and those meanings are not private. A daughter's silence means "respect" to her father, "punishment" to her mother, and "safety" to herself. The clinical task is not to find the true meaning but to make the several meanings audible to everyone in the room.

Claim 4 — Symptoms can serve the system. Not always, not deliberately, and never in a way that makes the symptomatic person responsible. But frequently enough that the therapist must ask: what would change in this household if this symptom disappeared tomorrow? Sometimes the honest answer is "nothing." Sometimes it is "my marriage would have to be looked at," and that answer reorganises the whole treatment.

Claim 5 — The observer is inside the observed system. You are not a camera. Your presence changes what the family does, what they say, what they notice about themselves, and what they will risk in front of each other. The family that arrives furious will perform its fury for you; the family that arrives polite will perform its politeness. This is not contamination to be minimised — it is the instrument you have. Second- order cybernetics, which students meet formally in MFT 715, is simply the discipline of using that instrument knowingly (Figure 1.1).

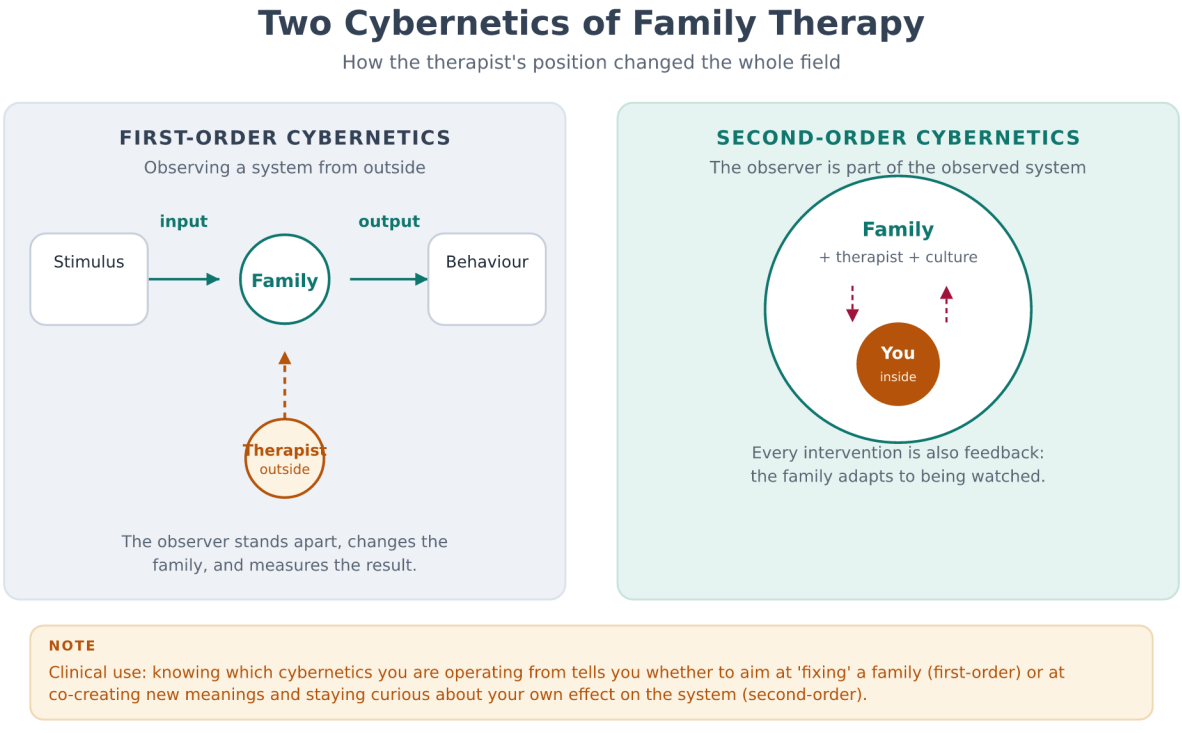


Figure 1.1 — First- and second-order cybernetics: where the therapist stands

A working definition. Marriage and family therapy is the assessment and treatment of relational systems — couples, families, and the wider networks in which they live — using interventions that target interaction patterns, meaning systems, and the emotional bonds between people. Read the definition carefully: assessment of relational systems means the unit of assessment is the relationship; interventions that target patterns means the unit of intervention is the loop, the rule, the structure or the story; emotional bonds means that a purely behavioural account is insufficient, because people are attached to each other and the attachment is what makes the pattern stubborn.

What it is not. Four clarifications, because students routinely collect the wrong ones.

1. **It is not "seeing everyone together" as a matter of policy.** Sometimes the right systemic move is an individual session inside family work. Conjoint work is a means, not a doctrine.
2. **It is not a denial that individuals exist, or that biology, trauma and temperament matter.** It is a disciplined focus on context, not a metaphysics.
3. **It is not the claim that families cause mental illness.** That claim was made in the 1950s and it was wrong, cruel, and expensive. Contemporary systemic practice holds families as the site where distress is expressed, maintained and relieved — not as the sole cause.
4. **It is not "we don't do diagnosis."** Systemic therapists diagnose, formulate and often work inside services that require a diagnosis; see MFT 707.

1.4 The four consequences, worked through

The four consequences in the original week note are worth more than a bullet list. Take them one at a time, each with what it changes on Monday morning.

Consequence 1 — The unit of assessment changes. You must hear from more than one person, and notice what happens between them. In practice this means the assessment cannot be completed in a single interview with one informant. It means disagreement between informants is data rather than noise: if the mother says the boy is defiant and the father says he is fine, the difference is the finding. It means you watch the room — who sits where, who answers for whom, who looks at whom before speaking, what happens to the volume when one person enters.

Consequence 2 — The unit of intervention changes. You intervene in sequences, rules, meanings and structures, not only in individual cognition. A cognitive intervention asks the depressed woman to examine the evidence for her belief that she is a burden. A systemic intervention asks what happens in the household in the forty-eight hours after she expresses that belief — and often finds that the expression triggers reassurance, that reassurance prevents any change in the division of labour, and that the division of labour is what is depressing her. Both are legitimate. The second is invisible from inside the first.

Consequence 3 — The definition of success changes. Success may be a family that argues well, not a family that never argues. This is the hardest consequence for students trained in symptom reduction, because it means the goal is not the absence of conflict but the presence of **repair**. A couple who can fight, rupture and come back together within an

hour is doing better than a couple who never fight and are quietly contemptuous. Outcome measures should reflect this: not only "has the symptom gone?" but "can this system handle disagreement without breaking?"

Consequence 4 — The therapist's position changes. You are not a technician outside the system; you are a participant who affects it. Three practical implications. First, you must account for your own effect: the family will behave differently because you are watching, and the difference is informative rather than annoying. Second, you cannot claim neutrality; you can only be transparent about your partiality. Third, your own family of origin is in the room with you — which is why every week of this book ends with a reflective task, and why personal therapy and supervision are professional requirements, not luxuries.

> **CASE**
> Mrs Okonkwo brings her 9-year-old son, Ebuka, for "stubbornness". In the first ten minutes she mentions, twice, that his father "is never home". The linear frame says: assess and treat Ebuka's behaviour. The systemic frame says: ask who else is affected by the behaviour, what happens after it, and what would change in the household if it stopped. The second question is not a rejection of the first; it is a widening of the field in which the first can be treated.

1.5 Systems as a way of thinking

Students often assume "system" is jargon for "family". It is not. A system is any set of elements whose interaction produces effects none of the elements produces alone. Ludwig von Bertalanffy's general system theory (1968), Norbert Wiener's cybernetics (1948) and the communications research at Palo Alto gave the field three tools, and they are still the three tools.

Feedback. A system regulates itself through loops. **Negative feedback** reduces deviation and restores the previous state — the family that pulls a member back whenever they move too far, the couple whose argument returns to the same sentence. **Positive feedback** amplifies deviation and produces change or escalation — the quarrel that grows, the affair that is discovered, the child who leaves. Note that the words are not evaluative: negative feedback is not bad, positive is not good. Most clinical problems are stuck negative-feedback loops with high cost; most crises are positive-feedback loops that have run away.

Homeostasis and morphostasis. Systems resist change in order to remain themselves. When a family comes to therapy, two processes are always running: the pressure to solve the problem and the pressure to keep the system intact. This is why families sometimes seem to sabotage their own treatment, and why the therapist who moves too fast loses the family. The technical answer is to pace: introduce change at a rate the system can absorb, and check that each change is stable before the next.

Rules and punctuation. Watzlawick, Beavin and Jackson's Pragmatics of Human Communication (1967) gave the field its most quoted propositions: one cannot not

communicate; every communication has a content and a relationship level; relationships are defined by punctuation of sequences; humans communicate digitally (words) and analogically (behaviour, tone, gesture). The idea of **punctuation** is the most clinically valuable. Each partner punctuates the same sequence differently: she says "he goes silent, so I get loud"; he says "she gets loud, so I go silent". Both are describing the same loop from different points on it, and each punctuation makes the other the cause. Therapy begins when the loop, rather than either partner, becomes the object of attention.

Structure. Minuchin added what communication theory lacked: an account of organisation. Families have subsystems (parental, sibling, couple), boundaries (clear, diffuse, rigid), hierarchies, alliances and coalitions. A child who is in a coalition with one parent against the other is not "manipulative"; the child is holding a structural position that the adults' conflict has created. Structure gives the therapist something to see in the seating plan and something to change in the session.

Meaning. The later schools — Milan, narrative, collaborative — pushed the field from pattern to interpretation. If the system is a set of interactions, it is also a set of stories about those interactions. A family that has decided "our son is the problem" has organised its attention, its conversations and its hopes around that sentence. Change the sentence and the interactions move; move the interactions and the sentence loses its evidence.

> **PRACTICAL**
> Choose an ordinary recurring household event: the school run, Sunday lunch, the nightly argument about homework. Draw it as a loop with four to six nodes, each node a behaviour, with arrows showing what follows what. Then mark: where is the negative feedback (what restores the previous state)? Where is the positive feedback (what escalates)? Where does each participant punctuate the loop? Bring the drawing to the lab; you will redraw it after hearing someone else's account of the same event.

1.11a What the evidence says, in one page

Students are often told either that family therapy "works" or that the evidence is contested. Both are unhelpful. Here is what a careful reading of the outcome literature supports, stated with the hedge each claim requires.

Family and couple therapy outperform no treatment, and generally match individual treatment for relational problems. Meta-analyses of couple and family therapy trials report effect sizes in the moderate range — roughly half a standard deviation to eight-tenths — against no-treatment and wait-list controls, with the larger effects for couple distress, child conduct problems, and adolescent substance use. Effects are smaller and less consistent against credible active comparators, which is what you would expect if the specific model matters less than the common structure of good therapy.

The model explains less of the variance than the alliance. The most robust finding in psychotherapy research, replicated across modalities, is that the therapeutic alliance predicts outcome better than the technical ingredients of any particular school. In couple

and family work the alliance is more complicated than in individual work, because there is more than one alliance to build and they can pull against each other: siding with one partner to engage them alienates the other, and a determined even-handedness can leave both feeling unheard. This is why Week 6 of this book spends two sessions on joining, and why your assessed skill in this course includes your ability to hold two alliances at once.

Common factors do a lot of the work. Client factors (severity, chronicity, social support, motivation), the alliance, the therapist's skill and the expectancy generated by a credible rationale account for a large share of change. The practical implication is not that technique is irrelevant but that technique is delivered through a relationship, and a technique delivered through a poor relationship is inert or harmful.

Dose and dropout matter more than most trainees think. A substantial proportion of families attend one session and never return; the first session is therefore an outcome event, not a preliminary. Retention is predicted by early alliance, by the fit between the therapist's explanation and the family's own understanding, and by practicalities — travel, cost, timing, childcare — that a therapist who ignores them will mistake for resistance.

Evidence is unevenly distributed across problems. Couple therapy for relationship distress, behavioural parent training for conduct problems, family-based treatment for adolescent eating disorders, multidimensional family therapy for adolescent substance use, and systemic work with chronic illness in children have reasonably strong support. Evidence for some popular ideas — mechanical use of paradox, unexamined "dysfunctional family" typologies, polyvagal explanations treated as established mechanism — is much weaker. Learning to distinguish these is part of professional formation, and MFT 705 and MFT 721 teach you to read the literature rather than the marketing.

> **CAUTION**
> Do not let the common-factors literature become an excuse for not learning a model. "The alliance is what matters" is true and is also the sentence most often used by trainees who have not yet learned to do anything. You need a model precise enough to be taught, supervised and evaluated, and a relationship good enough for the model to reach the family. Neither substitutes for the other.

1.6 First contact: three openings, annotated

The relational frame is not announced, it is enacted. Three openings show what it looks like when a therapist begins relationally in the first five minutes — before any genogram, any diagnosis, any technique.

Opening A — from the referred person to the referral system.

> **THERAPIST:**
> Before we start, tell me how this appointment came about. Who first said something needed to change, and who agreed? **Mrs Okonkwo:** The school called me. His teacher said he refuses to write anything. **Therapist:** And when you told your husband, what did he say? **Mrs Okonkwo:** (pause) He said I should handle it. He is not around. **Therapist:** So the school spoke to you, you spoke to your husband, and here you are with Ebuka. Who else in the family knows you are here today?

Note what the therapist has done. She has not asked about the symptom at all. She has mapped the **referral pathway**, which is itself a relational fact: who is worried, who is not, who carries the problem, who was told and who was not. She has also discovered, in four questions, the sentence that will organise the assessment: he said I should handle it. That sentence is not a complaint about a husband. It is a description of a household in which one adult holds all the worry, and a child's symptom sits in the space where the other adult would be.

Opening B — from the symptom to the sequence.

> **THERAPIST:**
> Tell me about the last time it happened. Not the worst time — the most recent. **Mrs Okonkwo:** Yesterday morning. He would not get out of bed. I called him three times, then I started shouting, then he got up and stood in the bathroom for twenty minutes. **Therapist:** And then? **Mrs Okonkwo:** Then I drove him late and I was angry all day. **Therapist:** What did Ebuka do when you were angry all day? **Mrs Okonkwo:** He was very quiet. He even made me tea.

The sequence is now visible: refusal, escalation, compliance, and then **repair initiated by the child**. That is the loop. The therapist did not need to interpret it. She asked for a specific episode and then asked "and then?" until the loop closed. This is the single most useful micro-skill in the first session: **episode, not opinion.** Opinions produce character descriptions ("he is stubborn"); episodes produce sequences.

Opening C — from the individual to the room.

> **THERAPIST:**
> (to Ebuka) Your mum says mornings are hard. If I asked her to describe a good morning, what would she say happens? **Ebuka:** (shrugs) She would be happy. **Therapist:** How would she know? What would she see? **Ebuka:** (long pause) My dad would be there.

Three questions, and the child has produced the formulation. The therapist did not ask "how do you feel about your father's absence?" — a question no nine-year-old answers truthfully. She asked a **behavioural question about a third person's perception**, which is easier to answer and much harder to fake. Involving the child as an informant, not a patient, is a structural act: it moves the child out of the identified- patient position and into the family's information system.

What all three openings have in common. Each one moves the unit of attention one step outward — from the person to the pathway, from the event to the sequence, from the individual to the room — and each does so with a question that is answerable. That is the whole craft of a first session in miniature. The student who can do these three moves in the first ten minutes will get a formulation. The student who cannot will get a history.

1.7 The Okonkwo case, fully worked

Referral. Ebuka, 9, referred by his school for "defiance and refusal to write". Six weeks of refusing to complete written work; two episodes of leaving the classroom. Class teacher describes him as "stubborn and rude". Mother reports he is "fine at home, except mornings". Father did not attend.

What the linear frame produces. A differential diagnosis: oppositional defiant disorder versus a specific learning difficulty (writing), versus anxiety, versus an attention problem. An assessment plan: psychometric testing, teacher rating scales, classroom observation, possibly a paediatric review. A treatment plan: behavioural contract, reward chart, parent training in commands and consequences. This is not wrong. It is incomplete in a way that will matter, because the most important information in this family is not about Ebuka's writing.

What the systemic frame adds. Four moves, in order.

Move 1 — the referral pathway. Who is worried? The school, then the mother. Who is not in the room? The father. Who decided that the mother should manage it? The father, according to the mother. The first structural fact: **the parental subsystem is not operating as a subsystem.** One adult holds the worry, the other holds the distance.

Move 2 — the sequence. Mornings: refusal, three calls, shouting, compliance, lateness, a day of maternal anger, evening reparation initiated by the child. The second structural fact: **the child regulates the mother's emotional state.** He absorbs her anger and then repairs it. A nine-year-old is doing the work of a partner.

Move 3 — the function of the symptom. What would change at home if Ebuka got up cheerfully and wrote fluently? The mornings would be calm; the mother's day would be easier; and the father's absence would become a topic rather than a background condition. There is no evidence that anyone wants the child to be symptomatic. There is good evidence that the symptom sits at the exact point where a marital conversation would otherwise occur.

Move 4 — the wider system. School, church, extended family. In this case the maternal grandmother is mentioned twice and the paternal aunt once. In many Nigerian households the grandmother is a co-parent and a court of appeal; leaving her out of the assessment is not neutrality, it is an omission.

The formulation, in one paragraph. Ebuka's refusal to write is maintained by a morning sequence in which escalation reliably produces compliance and in which the child then repairs his mother's distress. That sequence is itself maintained by a household in which one parent carries all the worry and the other is absent, and by a school system that has defined the problem as a defect in the child. The symptom is not caused by the

marriage, and nobody is to blame; but the symptom is located where the marriage would otherwise be discussed, and it will be difficult to resolve without that conversation becoming possible.

The plan that follows. Conjoint sessions including the father (with an explicit invitation framed as expertise rather than blame: "I need your view of what happens in the mornings; your wife and I are working from one account"). A behavioural component for the mornings, because it works and because success builds the parents' confidence in themselves as a team. A separate conversation with the parents about the marriage, held only when both are willing. Liaison with the school to reframe "defiance" as "a problem we are assessing", which reduces the pressure that feeds the loop.

> **TASK**
> Rewrite this case as you would present it in a multidisciplinary team meeting in forty seconds, without using the word "systemic" and without blaming either parent. Then rewrite it for the parents themselves. The two versions should differ in vocabulary, not in content.

1.8 Where the profession sits

Students enter training with a blurred map of the mental health professions. In Nigeria, as in most places, the practical differences matter for referral, for legal standing, and for what you are allowed to do.

Profession	Core unit	Typical training	What they usually do with a struggling family
Psychiatry	The patient, medically	Medical degree, then residency	Diagnose, prescribe, admit; refer for therapy
Clinical psychology	The individual	Doctorate or master's plus internship	Assess, formulate, deliver individual therapy
Counselling psychology	The individual in context	Master's or doctorate	Individual and group counselling, some couple work
Social work	The person in their environment	Degree plus registration	Case management, child protection, family support
Marriage and family therapy	The relationship	Master's or doctorate in MFT	Conjoint, family and couple therapy; relational formulation

The distinctive contribution is not a technique. Clinical psychologists do behavioural couple work; psychiatrists do family meetings; social workers do family support. The distinctive contribution is the **relational formulation** — the disciplined habit of locating the problem in the space between people, and the training in interventions that change that space. A therapist who can only see individuals will treat a family by treating its members one at a time and will be genuinely puzzled when the symptom returns.

Settings in which the work happens. Outpatient clinics and private practice; hospitals, where family meetings follow diagnoses that reorganise households; schools, where the referral pathway usually begins; NGOs and humanitarian settings, where family

separation, displacement and bereavement are the presenting context; faith settings, where pastors, imams and church counsellors are often the first and sometimes the only port of call; and courts and child protection services, where the therapist's opinion may be read by a judge. Each setting changes the ethics (see MFT 704), the assessment (MFT 707) and even the language you use.

Scope of practice. Three boundaries worth stating on the first day of training. First, family therapists do not prescribe; any question about medication is a referral question, and the referral should be made without implying that medication is a failure. Second, family therapists do not give legal advice; they provide clinical opinion and, where required, reports, and they tell families to obtain a lawyer (see MFT 724). Third, family therapists do not practise beyond their competence — and competence is specific: adult couple work is not child work, child work is not forensic work, and neither is trauma processing without specific training (MFT 711, MFT 714).

> **AUDIT**
> Write down, in one sentence each, what you would do and would not do if a family asked you to (a) advise whether the husband can legally take a second wife, (b) tell the court which parent should have custody, (c) prescribe something for the child's sleep. Bring the three sentences to the lab.

1.9 Culture, faith and class in the first meeting

The relational frame is not culturally neutral, and pretending otherwise is its own kind of arrogance. Three adjustments matter from the first session in most Nigerian consulting rooms.

Kinship is wider than the household. A genogram that stops at the nuclear family will miss the grandmother who raised the child for three years, the uncle who pays the school fees and therefore has standing, the aunt who lives with the family and disciplines the children, and the cousin whose presence has changed the sleeping arrangements. Ask who is involved, not who lives there. The distinction between household and family is one of the most practically useful in the whole programme.

Faith is a resource and a referral system, not a rival. Families commonly arrive having already seen a pastor, an imam, a traditional healer or all three. The therapist's task is not to adjudicate but to coordinate: what did the prayer, the deliverance, the fasting, the herbal treatment do? What was helpful? What was frightening? What does the family believe is happening spiritually? A family that believes the problem is spiritual and a therapist who believes it is relational can still work together if the therapist takes the belief seriously as a meaning in the system, which is exactly what it is.

Class shapes what is possible. A recommendation to reduce working hours is meaningless to a family with two jobs and no childcare. A recommendation to attend weekly sessions is meaningless to a family who travel four hours. A recommendation to "talk to your husband" is meaningless in a household where his mother is the decision-maker. Practical recommendations fail not because families are resistant but

because the therapist prescribed from a life the family does not have. The discipline is to ask, before prescribing: what would have to be true in your week for this to happen? The answer is usually the real treatment plan.

> **CAUTION**
> Do not treat cultural deference as consent. A wife who says "I will do whatever my husband decides" may be expressing a genuine value, or may be signalling that she cannot speak safely. The difference is discovered by seeing her alone at least once, and by asking about safety in the standard way — not by assuming either interpretation.

1.10 Six errors that begin in week one

These are the mistakes students actually make in their first role-plays, and each one is a failure of the relational frame rather than a failure of technique.

1. **The single-informant assessment.** You interview the mother, form a picture, and never test it. Fix: no relational formulation without at least two informants, and no family assessment without asking who is missing.
2. **The character formulation.** "He is oppositional." "She is enmeshed." These are descriptions of persons disguised as systemic language. Fix: every formulation must contain at least one sequence ("when X, then Y, then Z") and at least one context ("in the presence of…").
3. **Premature reassurance.** "I'm sure he loves you." It closes the exploration and hands the therapist's opinion to a family that needs its own. Fix: acknowledge the experience, ask for the evidence, keep the question open.
4. **The alliance slide.** You begin to like one member more than another and your questions begin to build the case for that person. Fix: track your questions across a session (record it) and count how many were addressed to each person and whose account each question elaborated.
5. **The technique-first session.** You arrive with a genogram to draw, a technique to demonstrate, a model to apply — and the family's actual concern is never heard. Fix: the first session belongs to the family's account, not to the therapist's plan.
6. **The safety omission.** You conduct a full conjoint assessment without ever asking about violence. Fix: every first session includes a private screen for harm to self, harm to others, abuse of children and intimate partner violence, conducted in a way that makes disclosure possible (see MFT 704 and MFT 712).

> **REFLECT**
> Which of the six are you most likely to commit? Not in theory: in your last conversation with someone who was struggling. Write the answer down. You will be asked to revisit it in Week 14.

1.11 Laboratory: the widening question

The week's practical is a triad drill in which the therapist may only ask questions that include another person. It looks simple and it is not: most students discover that their natural question is a question about an individual, and that widening feels artificial for the first twenty minutes.

Setup. Groups of three. Roles rotate every eight minutes, so each person is therapist once. One person brings a real but non-clinical difficulty — a disagreement with a flatmate, a difficult decision about a sibling, a strain with a colleague. No clinical material, no disclosure that would be regretted.

Rules for the therapist.

1. No statements, interpretations or advice — questions only.
2. Every question must include a third party or a relationship: "who notices…", "what does your sister do when…", "who would disagree…", "what happens between you after…".
3. One question at a time, with silence after each.
4. Eight minutes, then stop whether or not the conversation feels finished.

Observer's task. Two columns on one sheet: **W** (questions that widened the field) and **N** (questions that narrowed it back to the individual). Tally, then quote the single most useful widening question and the single most narrowing one.

Debrief questions. What did the client notice about their own description when it was widened? Which widening felt intrusive, and why? What did the therapist want to say but could not? What happened to the client's tone when a third person entered the story?

Marking rubric (used in the Week 6 skills gate).

Criterion	Weight	What full marks looks like
Proportion of widening questions	30%	More than 80% of questions include another person or a relationship
Question quality	25%	Open, specific, answerable; no double questions; no embedded advice
Pacing and silence	20%	Roughly one question per 30–45 seconds; client speaks more than therapist
Use of the client's words	15%	At least three questions reuse the client's own phrasing
Reflexivity in debrief	10%	Can name what was hard, what was tempting, what they avoided

> **DRILL**
> Record a five-minute role-play on your phone. Count your questions. Then count how many contain a third person. Most first attempts are around one in ten. The target is eight in ten, and it takes about three hours of deliberate practice to get there.

1.12 Reflective task

Write 500 words on the last significant conflict in your own family, then rewrite it from the point of view of the person you understood least. Note what changed in your body as you wrote. Keep both versions; you will return to them in Week 14, when you write your emerging clinical stance, and you will find that the second version contains the therapist you are becoming.

Two rules for this task. Do not show it to the person concerned, and do not bring the person's name into the seminar. The purpose is to develop the capacity to hold more than one punctuation of the same sequence at once — which is precisely what you will ask families to do.

1.13 Self-test

Six questions, answers below. If you cannot answer 4 and 5 without notes, reread 1.5.

1. Define negative and positive feedback, and give one clinical example of each.
2. Why is "punctuation" clinically useful?
3. State the double-bind hypothesis and its current scientific status.
4. What is symptom substitution, and what does it imply for treatment?
5. Name the five claims of the relational frame, in order.
6. What are the four consequences of treating the relationship as the client?

Answers. 1. Negative feedback reduces deviation and restores a prior state (a family pulling a member back from independence); positive feedback amplifies deviation (an argument that escalates through counter-complaint). 2. Because each participant's punctuation makes the other appear to be the cause; the therapist who makes punctuation visible turns blame into a loop both can act on. 3. A person receives contradictory messages at different logical levels, cannot comment on the contradiction and cannot leave the field; it is historically important and not a supported causal explanation of schizophrenia. 4. The observation that removing a symptom is often followed by a new symptom elsewhere in the system; it implies that treatment must change the system, not only the symptom. 5. Behaviour is patterned; patterns are maintained by consequences rather than causes; meaning is negotiated; symptoms can serve the system; the observer is inside the observed system. 6. The unit of assessment, the unit of intervention, the definition of success, and the therapist's position.

1.14 Readings for this week

Read the first two before the lecture; the rest are for the assignment.

- **Bateson, Jackson, Haley & Weakland (1956)**, "Toward a theory of schizophrenia", Behavioral Science. Read it as a historical document and notice how carefully the authors hedge.
- **Watzlawick, Beavin & Jackson (1967)**, Pragmatics of Human Communication, chapters 1–2. The axioms, in the original.

- **Nichols & Schwartz**, Family Therapy: Concepts and Methods, chapter 1. The best single overview of the field's history.
- **Becvar & Becvar**, Family Therapy: A Systemic Integration, chapters 3–5. On cybernetics and second-order thinking.
- **Hoffman (1985)**, "Beyond power and control", Family Systems Medicine. The turn to second-order cybernetics, written by a participant.
- **Minuchin (1974)**, Families and Family Therapy, chapter 2. Structure, in Minuchin's own voice.

1.6a An extended transcript, annotated

Role-plays teach faster than principles. Below is a twelve-turn excerpt from a first session with a couple who present with "we cannot communicate". The therapist is a trainee in her second term. Read the transcript, cover the annotations, and decide what you would have asked next. Then read the annotations.

> **T1 THERAPIST:**
> What would you like me to understand first? **T2 Folake:** He doesn't talk to me. He comes home, eats, watches television, sleeps. I am alone in this house. **T3 Therapist:** (to Dele) What do you think your wife means when she says she is alone? **T4 Dele:** I don't know. I work. I am tired. She is not alone, there are three children in the house. **T5 Therapist:** Folake, what do you do when he is watching television? **T6 Folake:** I stand there. Sometimes I ask him something and he says "mm". **T7 Therapist:** And what happens then? **T8 Folake:** I get angry. I say something sharp. Then he goes to bed. **T9 Therapist:** Dele, when she says something sharp, what goes through your mind? **T10 Dele:** That nothing I do is enough. So I stop. **T11 Therapist:** And when you stop, Folake, what happens to the anger? **T12 Folake:** It grows. And then I cannot sleep and I am tired the next day and the children see me crying.

Annotations.

T1 is a neutral opening that hands the agenda to the couple without inviting either partner to be the complainant. Compare "What is the problem?" — which usually produces one person's problem statement and positions the other as the problem.

T3 is the first systemic move: instead of asking Dele to defend himself, the therapist asks him to **interpret his wife**, which requires him to engage with her experience rather than his own. It also reveals whether he has a theory of her mind at all. He does not: "I don't know" is honest and important.

T5 is an **episode question** and it is doing three things at once. It bypasses the argument about whether she is alone (unwinnable), it specifies a moment (television, evening), and it puts the therapist physically in the room with them. Students who ask "why do you feel alone?" get an explanation; students who ask "what do you do when he is watching television?" get the behaviour that maintains the loneliness.

T7, "and then?", is the single most productive two-word intervention in family therapy. It builds the sequence one link at a time and stops the therapist from doing the formulating.

T9 takes the same event and asks for the **other partner's internal sequence**, which turns two monologues into one loop. Note that the therapist does not challenge Dele's account ("three children in the house" is not an answer to loneliness, but arguing about it would lose him). She lets the sequence do the work.

T10 is the pivot of the session. "Nothing I do is enough, so I stop" is a complete formulation of Dele's side of the loop, offered by Dele. Nothing the therapist could have said would have been as convincing or as usable.

T11 closes the loop by returning to Folake with the consequence of his withdrawal. At T12 she supplies it: the anger grows, sleep is lost, the children see her crying. The loop is now on the table, described entirely in the couple's own words, and neither partner has been blamed.

What the therapist has **not** done in twelve turns is worth listing, because trainees over-value these moves: she has not asked about the history of the marriage, has not asked how they met, has not asked about sex, has not offered an interpretation, has not normalised, has not asked either partner to change anything, and has not used the word "communication". Every one of those is legitimate; none of them was needed to establish the relational frame in the first ten minutes.

> **PRACTICAL**
> In pairs, take turns being the therapist with this transcript. Cover T3 to T12 and improvise your own next question at each point. Then compare with the original. Where did you ask for an opinion rather than an episode? Where did you reassure? Where did you take a side without noticing?

1.10a Three punctuations of one quarrel

The same sequence can be punctuated at least three ways, and each punctuation licenses a different intervention. Take the quarrel the couple above actually have.

Punctuation 1 — the character story. "He is cold and withdrawn; I am affectionate and he rejects me." What it licenses: an appeal to Dele to change, a diagnosis of Dele (avoidant, depressive, possibly autistic), and a treatment that works on Dele. What it costs: Folake's contribution disappears, Dele becomes a project rather than a partner, and Dele — who already believes nothing he does is enough — experiences therapy as one more arena in which he fails.

Punctuation 2 — the mirror story. "She criticises; I withdraw; she criticises more." What it licenses: a circular formulation, a sequence map, and interventions aimed at the loop (interrupting the escalation, slowing the exchange, rehearsing a different response to criticism). What it costs: the loop can become an abstraction that hides the asymmetry of power and the real losses — his hours, her sleeplessness, the children's exposure.

Punctuation 3 — the meaning story. "She is asking for evidence that she matters; he is protecting himself from evidence that he fails." What it licenses: work on attachment, on the meanings each assigns to the other's behaviour, and on the moment of turning toward or away. What it costs: meaning work without behaviour change can become insight that never reaches the household.

A competent therapist holds all three and chooses deliberately, usually sequencing them: the loop first (because it lowers blame), the meaning second (because it explains why the loop is so adhesive), and the character story never as a formulation, only as each partner's experience to be listened to. This pluralism — several punctuations available, one chosen for a reason — is what distinguishes a systemic clinician from someone who has memorised a model.

> **DEBATE**
> "Circular formulations are kinder but less true." Fifteen minutes. Note that the strongest argument for circularity is not moral but empirical: linear accounts of complex interaction are almost always wrong about causation, even when they are right about blame.

1.13a Examination question with model answer

Question (20 marks). "Family therapy is a way of seeing, not a set of techniques." Discuss, with reference to the historical development of the field and to at least one clinical example.

Model answer, in note form, with the markers' expectations visible.

Introduction (2 marks). State the claim in your own words: the relational frame is an epistemological commitment — a claim about the unit of analysis and the nature of causation — and techniques are downstream of it. Signal the structure of the answer: history, epistemology, clinical example, limits.

History (6 marks). Three research programmes converged on the same failure: Bateson's group and the double bind; Lidz and Wynne on family organisation around psychosis; the child guidance movement and symptom substitution. The aetiological claims of the 1950s were largely wrong and you should say so; what survived is the methodological insight that sequences carry information individuals do not. Trace the schools as answers to "what is the unit?" — Palo Alto (the sequence), Bowen (the generations), Minuchin (the structure), Milan (the meaning), Milwaukee (the solution), Adelaide (the story).

Epistemology (6 marks). Circular versus linear causality; feedback and homeostasis; punctuation; second-order cybernetics and the therapist's position. Use Watzlawick's axioms precisely — content and relationship levels, digital and analogic communication — and show what each buys clinically. Demonstrate that you understand the difference between an ecological claim (the system is the unit of assessment) and a causal claim (the system causes the symptom), and reject the second.

Clinical example (4 marks). The strongest answers use one case throughout rather than several in passing. The Okonkwo case works: a nine-year-old's school refusal, a morning

sequence, a parental subsystem that is not operating, and a formulation that no single-informant assessment could have produced. Show what the relational frame let you see and what it let you do.

Limits (2 marks). Name them honestly: the frame can obscure individual biology, trauma and temperament; it can be used to distribute responsibility in ways that leave an abusive member unaccountable; it can become a language that describes loops without ever changing them. Note the feminist critique of neutrality and the evidence that the alliance — not the model — predicts much of the outcome.

Common ways to lose marks. Describing techniques without explaining the frame; treating "systemic" as a synonym for "family"; repeating the double-bind hypothesis as if it were current; failing to give a clinical example at all; writing about circular causality without ever defining feedback.

1.13b What "good" looks like at the end of week one

A checklist you can use on yourself before Week 2. Tick only what you can do without notes.

- ■ Explain, in ninety seconds, why the field moved from the individual to the relationship, using three historical examples and one honest caveat about the science of the 1950s.
- ■ Define negative and positive feedback and give a clinical example of each that you have actually seen.
- ■ Explain punctuation to a couple without using the word "punctuation".
- ■ Turn three opinion questions into episode questions, out loud, without hesitation.
- ■ State the four consequences of treating the relationship as the client, and give one practical implication of each for a first session.
- ■ Describe the difference between a linear and a circular formulation using the same case twice.
- ■ Name the three professional boundaries from 1.8 and what you would say to a family that crossed them.
- ■ Ask about safety in a first session without it sounding like an accusation or an afterthought.

Week 2 — Linear and circular causality

The idea. Linear causality asks who started it and what caused it. Circular causality asks what maintains it and how each response feeds the next. This is the single most important conceptual shift in the course.

The shift is not merely philosophical. It changes what you do:

Linear question	Circular question
"When did the depression begin?"	"What happens in the house when she stays in bed?"

Linear question	Circular question
"Why does he lose his temper?"	"What does she do just before he raises his voice?"
"Who is to blame?"	"How does the blaming itself work?"

Punctuation. Each family member punctuates a sequence differently — she says he withdraws, he says she nags — and each believes the other started it. The therapist's job is not to decide who is right but to make the sequence visible so that both can see the same film (Figure 2).

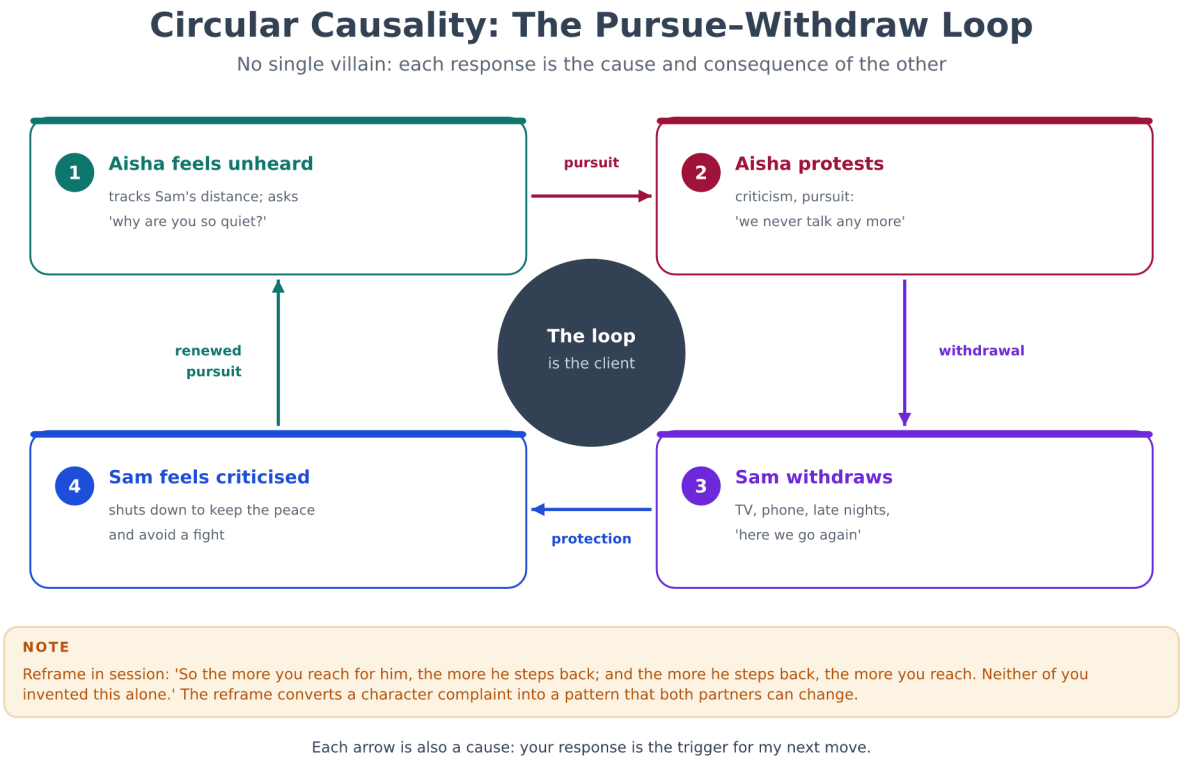


Figure 2.1 — The pursue–withdraw loop: a circular formulation

First-order and second-order cybernetics. First-order cybernetics treats the observer as outside the system: you measure, you intervene, you evaluate the result. Second-order cybernetics accepts that the observer is part of the system being observed: the family responds to being watched, and your presence is data. Contemporary practice is mostly second-order, which is why reflexivity (Week 10) is a clinical skill rather than a personality trait.

> **CASE**
> In a couple session, Folake says Dele "starts every fight". Dele says Folake "is never satisfied". Rather than adjudicate, the therapist asks each to describe, minute by minute, what happened last Tuesday. The sequence that emerges is: Folake raises a worry → Dele answers with a joke → Folake escalates → Dele leaves the room → Folake follows him → Dele goes silent for two days. Nobody started it. The loop started it.

Practice (lab). Watch five minutes of a recorded (consented) or scripted argument with the sound off. Write the sequence as a loop of four to six steps, with an arrow for each move. Then watch with sound and compare your loop with what the speakers said caused it.

> **KEY**
> If you can draw the loop, you can intervene in it. If you can only name a villain, you are still thinking linearly.

Week 3 — A short history of the field

You cannot critique a tradition you cannot narrate. This week gives you the story in five movements.

1. The pre-history (1900s–1950s). Child guidance clinics, marriage counselling, and social casework treated families, but individually. Psychoanalysis dominated, and the family was understood as the source of individual pathology rather than as a system in its own right.

2. The research rupture (1950s–1960s). Several independent groups — Bateson's Palo Alto project, Lidz and Wynne studying families of people diagnosed with schizophrenia, Bowen at Menninger, Ackerman in New York — began observing whole families and describing patterns rather than causes. Bateson's team gave us the double bind hypothesis and, more durably, the vocabulary of communication and paradox.

3. The heroic era (1960s–1970s). Charismatic founders built schools: Minuchin (structural), Haley and Madanes (strategic), Satir (experiential/human validation), Whitaker (symbolic-experiential), Bowen (multigenerational), Boszormenyi-Nagy (contextual), the Milan group (systemic/milan). Techniques were bold, sessions were long, and therapists were often filmed behind one-way mirrors.

4. The critique and the turn to meaning (1980s–1990s). Feminists (Hare-Mustin, Goldner, Walters) demonstrated that "neutral" systemic descriptions hid gendered power. Social constructionism and postmodernism (Anderson & Goolishian, White & Epston) shifted attention from structure to story. The Milan team's later work introduced curiosity and the reflecting team.

5. Evidence and integration (1990s–present). Manualised models were tested in randomised trials (behavioural couples therapy, EFT, multisystemic therapy, functional

family therapy, multidimensional family therapy). Attachment, neuroscience and trauma research entered the field. The current mainstream is integrative: model-informed, evidence-aware, culturally situated.

Why the history matters clinically. Every school was a reaction to a limitation of its predecessor. Structural therapy reacted to an over-focus on insight; narrative therapy reacted to an over-focus on expertise; trauma-informed care reacted to an over-focus on technique. When you find yourself irritated by a model, ask which limitation it is answering — you will usually find your own blind spot there.

> **PRACTICAL**
> Build a one-page timeline in small groups. For each decade, write (a) the dominant idea, (b) the model it produced, (c) the critique that ended its dominance. Present one decade each.

Week 4 — Systems concepts you will actually use

Six concepts do most of the clinical work. Learn them precisely; vagueness here produces vague therapy.

1. System and subsystem. A system is a set of elements in interaction such that a change in one produces a change in the others. Subsystems (the couple, the siblings, the parental dyad) are organised by boundaries.

2. Boundary. The invisible rules about who participates and how. Boundaries range from diffuse (enmeshed: little privacy, feelings shared before they are owned) to rigid (disengaged: autonomy without belonging). Healthy is not the middle of a line; healthy is appropriate to the task — the parental subsystem needs a clearer boundary than the sibling subsystem (Figure 5 in MFT 702).

3. Feedback. Negative feedback corrects deviation (a thermostat, a bedtime routine); positive feedback amplifies it (an escalation, a panic spiral). Families contain both, always. Symptoms often persist because a negative-feedback loop protects a positive-feedback loop underneath.

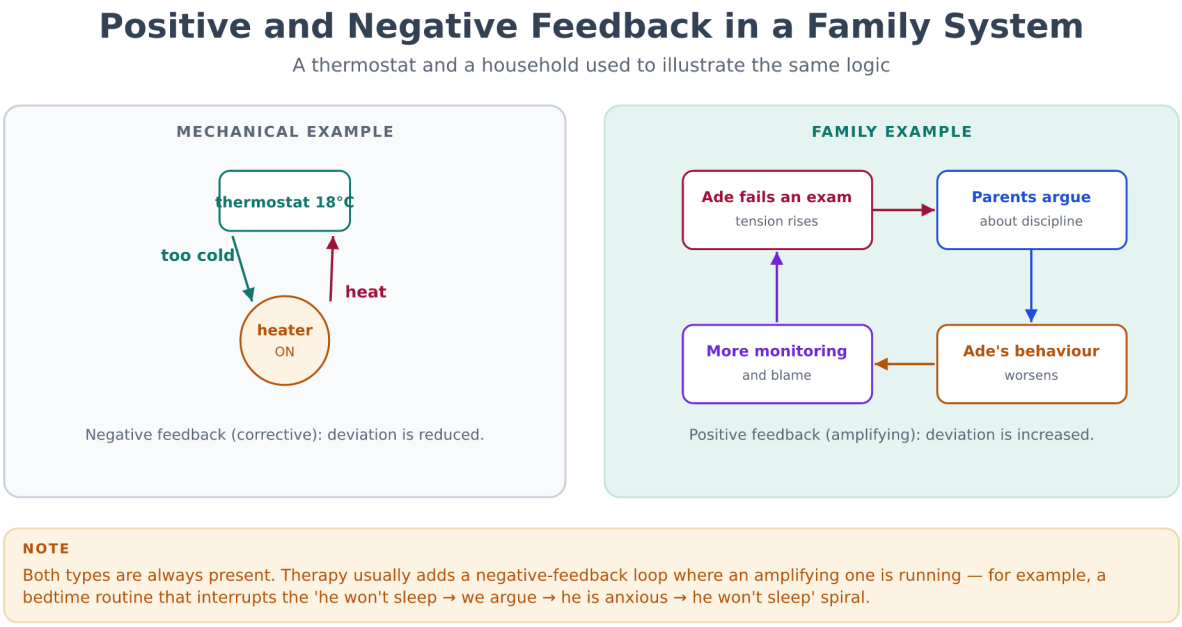


Figure 4.1 — The same feedback logic in a thermostat and in a family

4. Homeostasis. The system's tendency to maintain its familiar organisation, including familiar pathology. This is why a family may resist improvement: the drinking, the fighting, or the symptom has become load-bearing.

5. Rules. Recurrent patterns that prescribe and proscribe behaviour ("we do not discuss money"; "no one is allowed to be angry at Father"). Most family rules are never spoken; they are inferred from what produces anxiety.

6. Triangles. When tension between two people exceeds what the dyad can hold, a third is recruited — a child, an affair, a job, an in-law, a symptom. Triangles distribute anxiety; they also freeze the dyad's capacity to resolve it (covered fully in MFT 715).

> **CASE**
> Since the father's redundancy, the household has run a nightly cycle: he criticises the children's homework, the mother defends them, he accuses her of undermining him, she goes to bed early, and the eldest daughter stays up to calm him. Mapping it: the father's shame (unspoken) → criticism → mother's protection → his isolation → daughter's role as regulator. Nothing here is "the problem". The system has found a way to hold his shame — at the cost of a 14-year-old's adolescence.

Practice (lab). Using the case above, label each element: boundary, feedback type, homeostatic function, rule, triangle. Then answer: what would have to change elsewhere if the daughter stopped regulating?

Week 5 — The profession: identity, scope and settings

Family therapy is a distinct profession with its own training standards, codes and legal standing, and it is also a transdisciplinary competence. You need both facts.

Scope of practice. Assessing and treating relational distress; couple and marital therapy; family therapy with children and adolescents; parent guidance; separation and divorce work; work with stepfamilies; group work with relational focus; consultation to organisations. In most jurisdictions, family therapists do **not** prescribe medication, conduct forensic custody evaluations of their own clients, or give legal advice.

Settings. Private practice, community mental health, hospitals (including paediatric and psychiatric liaison), schools, child protection services, addiction services, employee assistance programmes, faith-based counselling centres, NGOs in humanitarian settings, and court-connected mediation.

Teamwork. You will work alongside psychiatrists, paediatricians, social workers, teachers, probation officers, imams and pastors, and community health workers. Two skills matter more than you expect: writing a referral that says what you want, and knowing the boundaries of what you may disclose.

Professional identity. Three tensions recur in the literature and in practice:

1. **Expert or collaborator?** The field has moved from therapist-as-director to therapist-as-curious- participant, but families often want expertise. The mature position is "expert in process, not in their lives".
2. **Neutral or advocating?** Systemic neutrality does not require moral neutrality about violence.
3. **Specialist or generalist?** You will be asked to work with issues you were not trained for. The professional answer is competence boundaries plus referral, not improvisation.

> **CASE**
> A school refers a 12-year-old for "aggression". The school wants an assessment letter; the mother wants the school to stop the bullying; the father does not want to attend. Assume nothing. Ask who defined the problem, who benefits from that definition, and what the referral is for. Then write down your scope: what you will assess, what you will not, and to whom you will send what.

> **PRACTICAL**
> Draft a one-page "Scope of practice" statement for yourself as an intern. Include the three things you will not do without supervision.

Week 6 — The first session I: joining

The idea. Joining is the process of becoming part of the system sufficiently to influence it, without becoming so much a part of it that you lose the capacity to observe. Minuchin called it the therapist's "accommodation" to the family's style. It is not friendliness; it is deliberate.

Four joining moves:

1. **Track the family's language.** Use their words, not your diagnostic ones. If they say "he is stubborn", ask about "stubborn" before you ask about "oppositionality".
2. **Validate what is true in each person's account.** Not "you are right", but "that makes sense given what you have been carrying".
3. **Match pace and formality.** Some families want a structured interview; others need ten minutes of narrative before any structure is tolerable. Ask: "How would you like to use this hour?"
4. **Make the structure explicit.** Who is here, why, for how long, who knows, what is confidential. Anxiety drops when the frame is clear.

What to cover in a first session (the "minimum viable first session"):

Element	Sample wording
Contract and limits	"I will keep what is said here private, except where someone is at risk of harm."
Each person's view	"I would like to hear from each of you — what worries you most?"
The attempted solutions	"What have you already tried? What happened?"
The pattern	"So when X happens, what does Y do? And then?"
Risk screen	"Is anyone being hurt at home? Any thoughts of ending your life?"
Feedback	"Here is what I think I heard. What have I got wrong?"
Next step	"I would like to see all of you again next week, including your father."

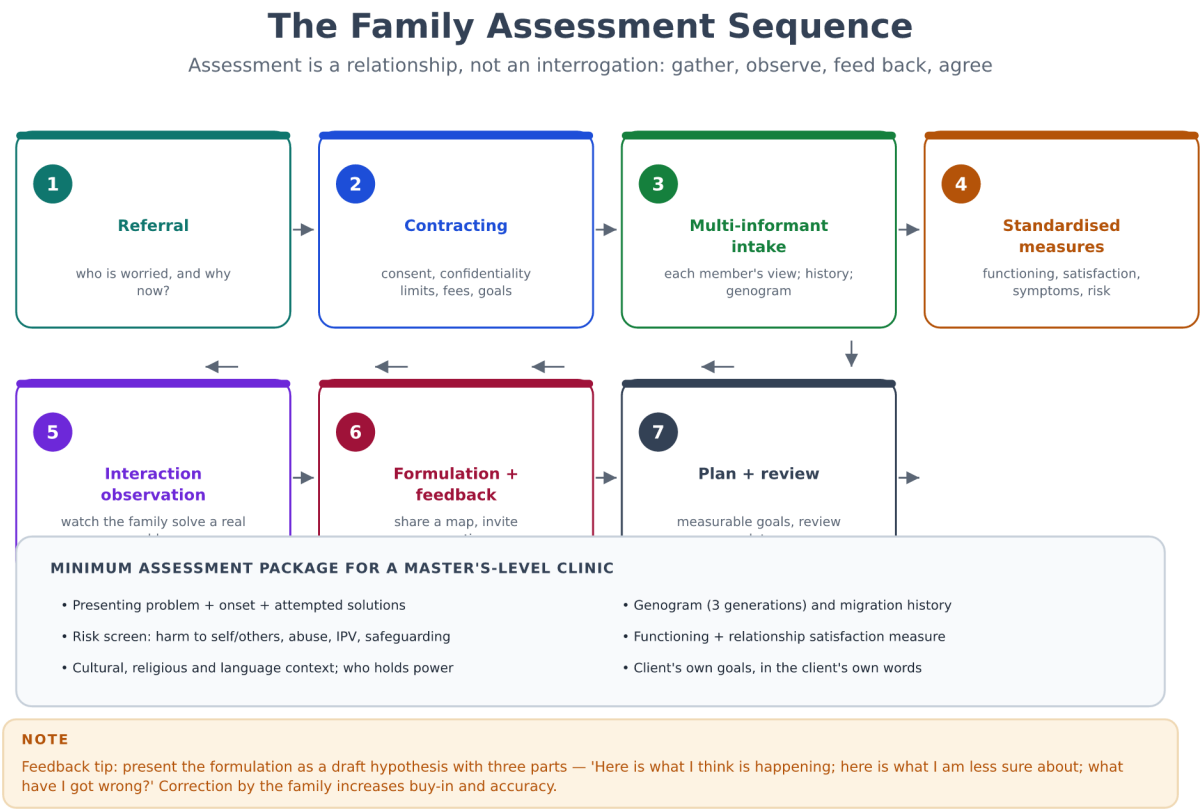


Figure 6.1 — The assessment sequence: assessment is a relationship, not an interrogation

CASE

A family arrives furious with each other and with the referrer. The therapist's first sentence is not a question but an acknowledgment: "You have been sent here by three different people and none of them asked you what you wanted. Let's start there." Joining succeeded before any information was gathered.

PRACTICAL (SKILLS GATE)

Fifteen minutes, observed. You must (1) state confidentiality and its limits, (2) obtain a description of the problem from at least two people, (3) ask one circular question, (4) complete a risk screen, (5) offer a summary and invite correction. You may use one index card of prompts. Feedback uses the four-point structured sheet in the programme handbook.

Week 7 — The first session II: defining the problem

The referral question is rarely the clinical question. Your task is to negotiate a problem definition that is (a) specific, (b) interactional, (c) owned by the family, and (d) changeable.

From vague to specific. "He is disrespectful" → "When I ask him to help, he looks at his phone and says nothing. That happens most evenings, and then I shout."

From individual to interactional. "She is manipulative" → "When she cries, her father gives in and you feel undermined; then you withdraw, and she comes to me." Notice: no blame is removed, but the description now includes three people and a sequence.

From imposed to owned. Ask each person to complete: "I will know this has worked when…". Write the answers down and read them back. If the parents' answers are incompatible, that incompatibility is the work.

From unchangeable to changeable. "I want him to respect me" is a relational outcome that no one can deliver on demand. "I want us to agree on one consequence for phone use, and both apply it for a week" is testable.

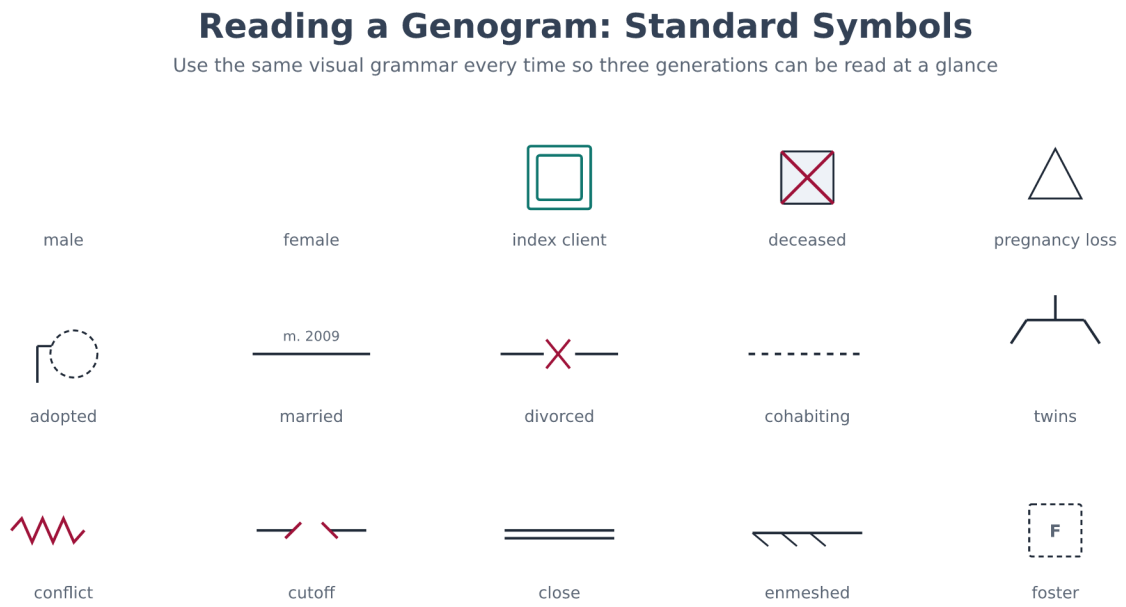
> **CASE**
> The Adeyemis come because their 16-year-old, Amara, has refused school for six weeks. The referral says "school refusal". The negotiated definition becomes: "The mornings have become a battle that everyone loses; Amara does not leave the house; her mother stays home from work; her father works later; and her brother has started sleeping in her room." The problem is now a system, not a girl.

Practice (lab). Take a complaint a peer brings from placement (disguised) and rewrite it four times: specific, interactional, owned, changeable. Read the final version to the "family" and ask whether it is recognisable.

Week 8 — The genogram as a hypothesis machine

The genogram is a three-generation map of family structure, relationships and events. Drawing it is simple; using it is not — the value lies in the hypotheses it generates and the questions it permits.

Standard symbols (Figure 3) and conventions: squares for males, circles for females, horizontal line for marriage, slash for divorce, cross for death, double outline for the index client. Relationship lines: double for close, zigzag for conflict, broken with slashes for cutoff, triple for enmeshed.



NOTE

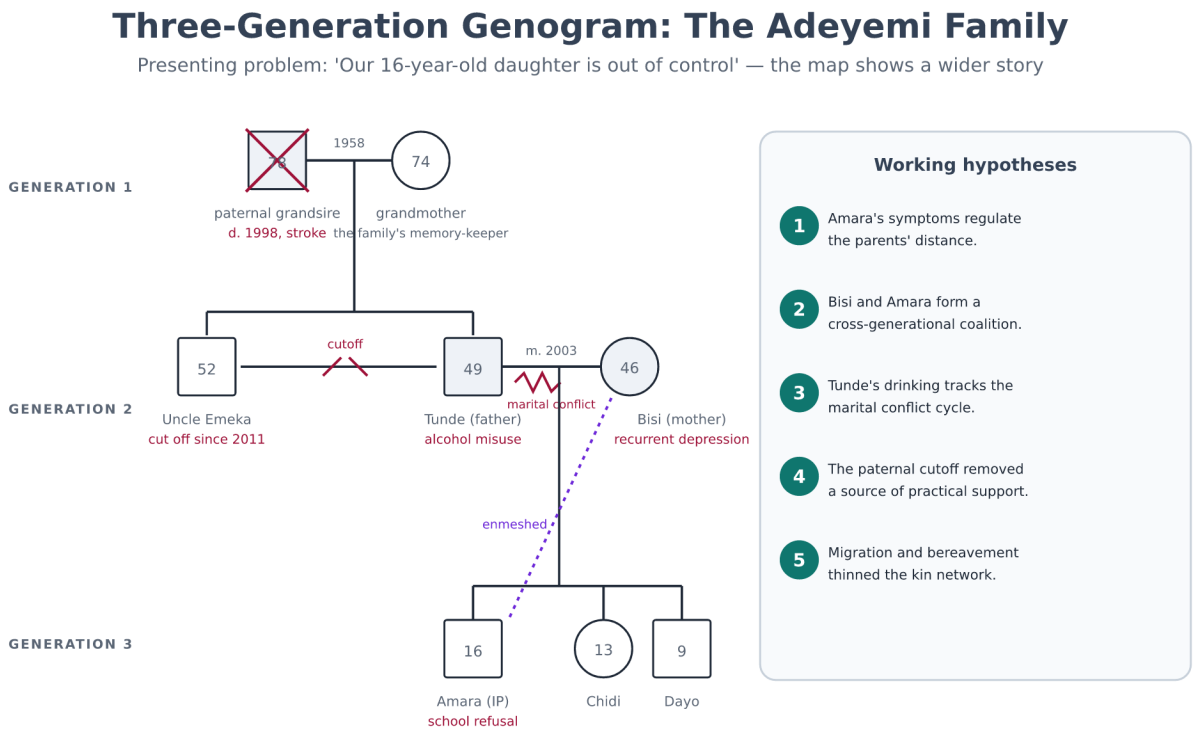
Also record: ages or year of birth inside the symbol, date and cause of death above it, occupation, migration, religion, and significant secrets (affairs, incarceration, abuse) using a key that you explain to the family before you draw.

Figure 3 — Genogram symbols: learn the grammar once and three generations become readable

What to ask while drawing. Age and year of birth; occupation and migration; deaths and their causes; separations, divorces, remarriages; who is close to whom; who does not speak to whom; who is the "responsible one"; who "had a breakdown"; family secrets; repeating names; repeating ages at which things happened.

From map to hypothesis. A good genogram yields three kinds of hypothesis:

1. **Structural** — coalitions, cutoffs, over/under-functioning pairs.
2. **Temporal** — patterns repeating at the same age, or symptom onset clustering around anniversaries.
3. **Legacy** — loyalty, indebtedness, roles inherited ("the eldest daughter always cares for everyone").



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 8.1 — A worked three-generation genogram with hypotheses

Cultural note. In many families the genogram must include people who are not biologically related but are structurally central — a foster child, a co-wife, a pastor, a long-term house help. Ask, "who else belongs to this family?" before you decide the map is complete.

PRACTICAL

Draw your own three-generation genogram from memory first, then correct it with a relative. Note every place where your memory and theirs differ. Those differences are the assignment.

Week 9 — Formulating relationally

A formulation is a provisional, testable explanation that connects the presenting problem to the system that maintains it, and points to where you would intervene.

Use the 5-P grid (presenting, predisposing, precipitating, perpetuating, protective) and add two relational columns: who is involved and what function the symptom serves.

Domain	Adeyemi family
Presenting	Amara, 16, school refusal × 6 weeks; parental conflict over discipline
Predisposing	Maternal depression; paternal alcohol misuse; migration severed the extended-family buffer
Precipitating	Father's redundancy; best friend left the city; transition to senior secondary
Perpetuating	Pursue–withdraw cycle between parents; mother–daughter coalition; avoidance rewarded by anxiety relief; disrupted sleep
Protective	Both parents attend; church community; Amara's engagement with art; an aunt who is trusted

The intervention sentence. End every formulation with one sentence of the form: "Given X, Y and Z, the most useful focus now is W, because it is the most changeable and the most load-bearing." For the Adeyemis: "Given the coalition and the parental distance, the most useful focus now is the couple's conflict cycle, because it is what the symptom is currently regulating."

Share it. A formulation that is never spoken is a private theory. Offer it as a draft with three parts: here is what I think, here is what I am unsure about, here is what I need from you.

> **ASSIGNMENT (30%)**
> 2,000 words plus a genogram. Choose a disguised or composite family from placement or a detailed role-play. Include: genogram, 5-P grid, circular formulation with a diagram, three competing hypotheses, one intervention plan with rationale, and a reflection on your own reactivity to this family.

Week 10 — Culture, class and power

Every family you meet is embedded in a culture, and so are you. Culture is not a variable to be "controlled for"; it is the medium in which the problem is defined.

Four practical disciplines.

1. **Ask rather than assume.** "In your family, who usually makes the big decisions? Who is consulted? Is that how you want it to be?"
2. **Locate yourself.** Complete the same cultural genogram for your own family. Your views on hierarchy, individuation, duty and disclosure are not neutral.
3. **Notice the power in the room** — including yours. The family may be polite because of your status, not because they agree.
4. **Interrogate "normal".** Models that prize individuation can pathologise families who prize interdependence. Ask what the family's own picture of a good life is.

Language. If the family thinks in their first language about emotional matters, the session may need that language or an interpreter. Interpreters are not furniture: brief

them, seat them beside you, ask them to interpret speech not summaries, and never ask them to translate a disclosure you have not prepared for.

Religion and spirituality. In much of the world, faith is not an "aspect" of family life; it is the organising frame. Ask what the family's faith says about the problem, who in the religious community knows, and whether they want that person involved. Do not compete with the pastor or imam; coordinate.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent twice. She does not want the police involved and does not want her family told. The therapist must hold three things at once: respect for her autonomy and faith, a safety plan that does not depend on him changing, and honesty about the limits of confidentiality. (Full treatment in MFT 712 and 724.)

> **REFLECT**
> Where in your own family would a therapist from a different culture misread the structure? Write 300 words on what they would get wrong and why.

Week 11 — What the evidence says

Students often arrive believing either that therapy "just works" or that only manualised treatments do. The evidence is more interesting than either position.

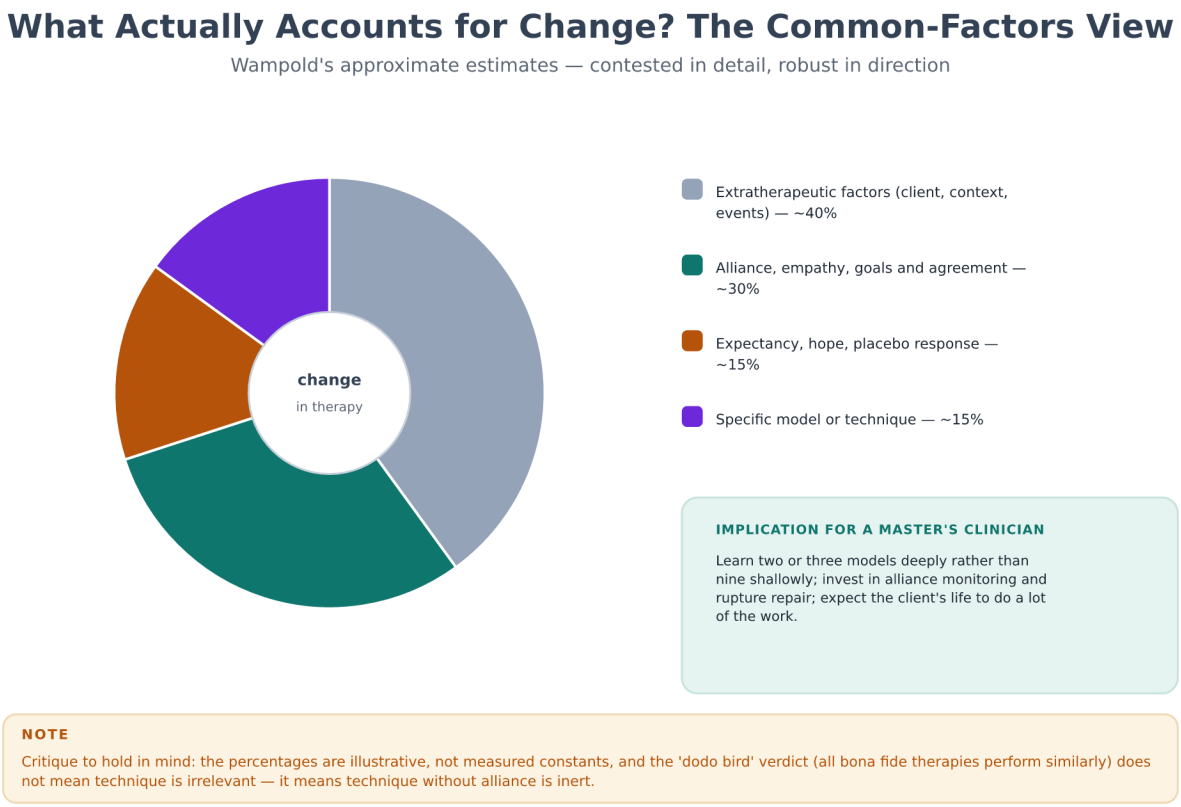


Figure 11.1 — Common factors and model effects, with a critique

Four findings worth carrying into practice.

1. **Family and couple therapies work.** Meta-analyses consistently find moderate-to-large effects for couple therapy in relationship distress, and for family-based treatments in adolescent behaviour problems, eating disorders (family-based treatment for anorexia is a first-line intervention), substance misuse and psychosis relapse.
2. **Alliance matters, and it is relational.** In couple and family work, the alliance is not one relationship but several, and shared sense of purpose predicts outcome better than any single dyad's alliance with the therapist.
3. **Model differences are smaller than model loyalties suggest.** Common-factors research attributes much of the variance to client factors, alliance and expectancy. Technique is not irrelevant: it organises what the therapist notices and does, and it matters more for specific problems.
4. **Drop-out is the silent failure.** In couple and family work, 20–40% of families end therapy unilaterally. Attendance, engagement and early change are the best predictors of retention. Track them.

How to read a paper in ten minutes. Read the abstract; then the design (who, how many, compared to what); then the outcome measure (does it measure what you care about?); then the effect size, not the p-value; then the limitations; then ask whether the sample resembles your clinic.

> **CRITICAL READING**
> Choose one randomised trial of a family therapy model and one qualitative study of family therapy experience. Write 500 words comparing what each can and cannot tell you.

Week 12 — Ethics, risk and the limits of the systemic frame

Ethics is not a chapter at the end of the course; it is the frame that makes the work possible. Here we establish the four issues every family therapist meets in the first month.

1. Confidentiality in a multiperson system. There is no single confidentiality. Agree the rules at the start: what is said in the joint session is shared; what is told to you individually may need to be brought into the open; secrets that affect safety cannot be kept. Write the policy down and give it to the family.

2. Risk. Screen for suicide, self-harm, violence, abuse and neglect in every new case, and document it. You cannot assume a colleague has done it.

3. Competence. Taking a case you are not trained for is not courage. It is a risk to the client and to you.

4. Multiple relationships and power. Small communities, religious networks and family ties make dual relationships likely. The test is not "is there a second relationship?" but "does the second relationship impair the first, or exploit the first?"

> **CASE**
> During an individual session, a husband tells you he has resumed an affair and asks you not to tell his wife. He is not at risk and no one is being harmed in the sense that requires disclosure. What do you do? (Work this case in groups; then compare with the decision model in Figure 33, MFT 704.)

Documentation. Write notes as if a colleague will take over tomorrow and a court may read them in a year: date, attendees, presenting issues, risk screen result, interventions, plan, and who was told what.

Week 13 — Integration I: three lenses on one family

This week is a rehearsal for MFT 702. You will apply three models to the same case and notice that each produces a different question, a different focus and a different first move.

Model	Key question	First move with the Adeyemis	Likely risk
Structural	What is the hierarchy, and where are the boundaries?	Strengthen the parental subsystem; block the mother–daughter coalition	Imposing a structure the family experiences as foreign

Model	Key question	First move with the Adeyemis	Likely risk
Bowen/multigenerational	How does anxiety travel, and who absorbs it?	Coach the parents to manage their own reactivity; map the projection onto Amara	Slow; family may want symptom relief now
Solution-focused	What is already working, and what would "one point better" look like?	Find exceptions to school refusal; scale the parents' confidence	May bypass the function of the symptom

The point of the exercise is not to choose a winner. It is to notice that a family presents a plural reality and that your model determines what you can see. Good clinicians can move between at least two frames without losing coherence.

> **PRACTICAL**
> In groups, take the same case and produce three one-page plans. Present all three. Then answer: which would you start with, and what would make you change your mind?

Week 14 — Integration II: your emerging clinical stance

Write a two-page **position statement**: what you believe about how relational problems develop, what you think therapy does, what you will not do, and what you still do not know. This is not an exam answer; it is the beginning of a professional identity that will be revised for the rest of your career.

A strong statement includes: one theoretical commitment, one epistemological commitment (how you know what you know), one ethical commitment, one cultural commitment, and one honest limitation.

> **REFLECT**
> Read your Week 1 reflection again. What has changed? What has not? Write 300 words to yourself, dated, to be reread at the end of the programme.

Integration: the whole course on two pages

1. The frame. Treat the relationship as the client. Widen the field; include context; notice the observer.

2. Causality. Move from who-started-it to what-maintains-it. Draw the loop. Punctuate differently.

3. History. Five movements: pre-history, research rupture, heroic era, critique and meaning, evidence and integration. Each school answers a limitation of its predecessor.

4. Concepts. System, subsystem, boundary, feedback, homeostasis, rule, triangle. Six concepts, most of the clinical work.

5. Profession. Know your scope, your setting, your team, your limits. Refer well.

6. The first session. Join, contract, hear everyone, screen for risk, map the pattern, summarise, invite correction, agree next step.

7. The problem definition. Specific, interactional, owned, changeable.

8. The genogram. Three generations, structural and temporal hypotheses, cultural completeness.

9. Formulation. 5-P plus "who is involved" and "what function the symptom serves", ending with one intervention sentence.

10. Culture and power. Ask, locate yourself, notice power, interrogate normal.

11. Evidence. Family therapy works; alliance matters; models overlap; drop-out is the silent failure.

12. Ethics. Confidentiality is negotiated, risk is screened, competence is bounded, dual roles are examined, notes are written for strangers.

13. Integration. Multiple lenses; move between frames without losing coherence.

14. Stance. A written, revisable position on what you believe and what you do not know.

Glossary

Boundary — the implicit rules governing participation and closeness between subsystems. **Circular causality** — an account in which each element both causes and is caused by the others. **Coalition** — an alliance of two or more people against a third. **Cybernetics** — the study of control and communication in systems; first-order (observer outside), second-order (observer inside). **Detriangulation** — the process of extricating a third party from a dyadic conflict. **Enmeshment** — diffuse boundaries with excessive emotional sharing and low autonomy. **Disengagement** — rigid boundaries with little emotional contact. **Feedback (negative)** — a process that reduces deviation and restores stability. **Feedback (positive)** — a process that amplifies deviation and produces escalation. **Genogram** — a three-generation diagram of family structure, relationships and events. **Homeostasis** — the system's tendency to maintain its existing organisation. **Hypothesis** — a provisional, testable explanation that guides enquiry. **Joining** — the therapist's deliberate accommodation to a family's style in order to work with it. **Linear causality** — an account in which A causes B in one direction. **Punctuation** — the way each participant marks the beginning of a sequence, thereby assigning blame. **Reframe** — relabelling a behaviour so that it is understood in a new, usually relational, frame. **Subsystem** — a unit within a system organised by boundaries (couple, siblings, parental dyad). **System** — a set of interacting elements such that a change in one changes the others. **Triangle** — a three-person configuration that absorbs and stabilises dyadic tension.

Self-assessment

Multiple choice (answers below)

1. A therapist says, "The real question is not who started the argument but what keeps it going." This statement reflects: (a) linear causality (b) circular causality (c) psychodynamic causality (d) moral causality

2. A father's drinking has the effect of preventing his wife and son from discussing the son's expulsion. In systemic terms the drinking is: (a) random (b) homeostatic (c) an individual symptom only (d) irrelevant to the family

3. "We never discuss money" is best described as: (a) a boundary (b) a family rule (c) a triangle (d) a reframe

4. In second-order cybernetics the therapist is: (a) outside the system (b) part of the observed system (c) a neutral observer (d) irrelevant

5. The best first move with a family that arrives furious at the referrer is: (a) correct the misunderstanding immediately (b) acknowledge their experience of being sent (c) begin the assessment (d) ask who is the identified patient

6. A negotiated problem definition should be specific, interactional, owned and: (a) clinically imposed (b) changeable (c) blame-allocating (d) historical

7. Which is not an element of a minimum first session? (a) confidentiality and limits (b) a risk screen (c) a full genogram (d) a summary and invitation to correct

8. The strongest predictor of drop-out in couple and family work is: (a) diagnosis (b) therapist's model (c) early engagement and early change (d) family size

9. "Multigenerational transmission" belongs to which tradition? (a) structural (b) Bowen (c) Milan (d) solution-focused

10. The statement "expert in process, not in their lives" describes: (a) neutrality about violence (b) the therapist's stance (c) a legal requirement (d) a diagnostic rule

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 c · 8 c · 9 b · 10 b

Short answer (150 words each)

1. Explain the difference between negative and positive feedback using a family example.
2. Why is "punctuation" clinically useful? Give a sample question.
3. Name three limits of confidentiality in family work and how you would state each to a family.
4. What is the difference between a genogram and a formulation?
5. Summarise the common-factors critique of model loyalty in four sentences.

Essay questions (choose one, 1,500 words)

1. "Family therapy is a frame, not a technique." Discuss with reference to at least two models and one case from your placement.
2. Critically evaluate the claim that systemic neutrality is impossible — and undesirable — in cases of domestic violence.
3. Using a disguised case, demonstrate how a linear formulation and a circular formulation would lead to different treatment plans. Which would you choose and why?

Practical and clinical log requirements

For each session you observe or conduct, record:

Field	Requirement
Date and duration	Exact
Attendees	Relationship to index client; ages; who was absent
Referral source and question	In the referrer's words
Presenting problem	In each attendee's words
Risk screen	Completed Y/N; result; action
Pattern observed	One loop or sequence, described behaviourally
Intervention	What you did, and why
Response	What happened next
Supervision	Who, when, what was discussed
Plan	Next step, with review date

The log is assessed for **specificity and consistency**, not for how impressive the case was. A log of ordinary sessions written precisely will score higher than a dramatic case written vaguely.

Further reading

Core texts

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
- Dallos, R., & Draper, R. (2015). An Introduction to Family Therapy (4th ed.). Open University Press.

Foundations and history

- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Bateson, G. (1972). Steps to an Ecology of Mind. Chandler.
- Hoffman, L. (1981). Foundations of Family Therapy. Basic Books.
- Gurman, A. S., Lebow, J. L., & Snyder, D. K. (Eds.). (2015). Clinical Handbook of Couple Therapy (5th ed.). Guilford.

Practice and technique

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Haley, J. (1980). Leaving Home. McGraw-Hill.
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Culture and critique

- McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.). Guilford.
- Hare-Mustin, R. (1978). A feminist approach to family therapy. Family Process, 17(2), 181–194.

Evidence

- Wampold, B. E., & Imel, Z. E. (2015). The Great Psychotherapy Debate (2nd ed.). Routledge.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Shadish, W. R., & Baldwin, S. A. (2003). Effects of family and marital psychotherapies: A meta-analysis. Journal of Consulting and Clinical Psychology, 71(6), 1080–1097.

Codes and standards

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- International Association of Marriage and Family Counselors. Ethical Code.

Next: MFT 702 — Theories of Family Therapy